

OUTPATIENT MEDICAL REPORT

Riverside Family Medicine & Internal Medicine

FICTIONAL TRAINING REPORT - NOT A REAL MEDICAL RECORD

Patient Name	Johny S.	MRN	RMF-TEST-20481
Date of Birth	14-Aug-1989	Age / Sex	37 years / Male
Date of Visit	24-Sep-2026	Visit Type	Outpatient follow-up
Attending Physician	Dr. Emily Carter, MD	Department	Internal Medicine

CHIEF COMPLAINT

Follow-up for elevated blood pressure and review of recent laboratory results. Patient also reports intermittent heartburn after heavy meals and seasonal nasal congestion.

HISTORY OF PRESENT ILLNESS

Johny S. is a 37-year-old male presenting for a routine internal medicine follow-up. He reports home blood pressure readings averaging approximately 145-150/90-94 mmHg over the past month. He denies chest pain, syncope, palpitations, focal weakness, or shortness of breath. He reports increased thirst at times but denies polyuria, vomiting, or abdominal pain. He describes burning epigastric discomfort and sour taste two to three evenings per week, usually after spicy or late meals. He also reports recurrent sneezing and nasal congestion during seasonal changes. There is no current fever, cough, wheezing, or acute respiratory distress.

PAST MEDICAL HISTORY

Essential hypertension diagnosed in 2024. Prediabetes noted previously; recent laboratory results now meet criteria for diabetes in this fictional scenario. Mixed hyperlipidemia documented in prior screening. No known cardiovascular disease, chronic kidney disease, or prior stroke.

FAMILY HISTORY

Father with hypertension and hyperlipidemia. Mother with allergic rhinitis. No known family history of premature myocardial infarction.

ALLERGIES

Drug allergies: No known drug allergies (NKDA). Food allergies: None reported.

CURRENT MEDICATIONS BEFORE THIS VISIT

Medication	Strength	Directions	Adherence
Amlodipine	5 mg	1 tablet once daily	Taking regularly
Metformin	500 mg	1 tablet with evening meal	Started 2 weeks ago

VITAL SIGNS

BP	148/92 mmHg	Pulse	82 bpm	Resp.	16/min
Temp.	98.4 F	SpO2	98% RA	Weight	86.4 kg
Height	176 cm	BMI	27.9 kg/m2	Pain	2/10

PHYSICAL EXAMINATION

System	Findings
General	Alert, oriented, cooperative, in no acute distress.
HEENT	Mild bilateral nasal mucosal edema. Oropharynx clear. No tonsillar exudate.
Cardiovascular	Regular rate and rhythm. No murmurs, rubs, or gallops. Peripheral pulses palpable.
Respiratory	Lungs clear to auscultation bilaterally. No wheezes, crackles, or rhonchi.
Abdomen	Soft, non-distended, non-tender. No guarding or rebound.
Neurologic	Grossly intact. No focal motor or sensory deficit noted.
Extremities	No edema or calf tenderness.

LABORATORY RESULTS (18-Sep-2026)

Test	Result	Unit	Reference Range	Flag
Fasting glucose	156	mg/dL	70-99	H
HbA1c	7.4	%	4.0-5.6	H
Total cholesterol	232	mg/dL	<200	H
LDL cholesterol	152	mg/dL	<100	H
HDL cholesterol	39	mg/dL	>=40	L
Triglycerides	205	mg/dL	<150	H
Creatinine	1.0	mg/dL	0.7-1.3	Normal
eGFR	94	mL/min/1.73m2	>=60	Normal
ALT	29	U/L	7-56	Normal
TSH	2.1	mIU/L	0.4-4.0	Normal

ASSESSMENT / DIAGNOSIS

The following diagnoses are fictional and are included to create a realistic ICD-10-CM coding test case.

Priority	Diagnosis	ICD-10-CM	Clinical evidence in this synthetic record
1	Essential (primary) hypertension	I10	BP 148/92 mmHg; elevated home readings; no secondary cause documented.
2	Type 2 diabetes mellitus without complications	E11.9	Fasting glucose 156 mg/dL and HbA1c 7.4%; no complication documented.
3	Mixed hyperlipidemia	E78.2	Elevated total cholesterol, LDL and triglycerides with low HDL.
4	Gastro-esophageal reflux disease without esophagitis	K21.9	Intermittent heartburn and sour taste; no esophagitis documented.
5	Allergic rhinitis, unspecified	J30.9	Seasonal sneezing and congestion; no more specific trigger documented.

PLAN

1. Continue amlodipine with dose adjustment as documented below; monitor home blood pressure twice daily for 14 days.
2. Continue metformin with meals; reinforce adherence and dietary measures; repeat HbA1c in approximately 3 months.
3. Begin lipid-lowering therapy and repeat fasting lipid panel after the treatment interval.
4. For reflux symptoms, use the medication listed below and avoid late meals, large meals, and known trigger foods.
5. Use the listed antihistamine for seasonal nasal symptoms; reassess if symptoms persist or become year-round.
6. Routine diabetic monitoring to include foot examination at follow-up and consideration of an eye examination referral. No acute complication documented today.
7. Return visit planned in 4 weeks for blood-pressure review, or sooner for new/worsening symptoms.

PRESCRIPTION / MEDICATION ORDERS

Medication	Dose	Sig	Route	Duration / Quantity
Amlodipine	10 mg	Take 1 tablet once daily	PO	30 days / 30 tablets
Metformin ER	500 mg	Take 1 tablet twice daily with meals	PO	90 days / 180 tablets
Atorvastatin	20 mg	Take 1 tablet nightly	PO	90 days / 90 tablets
Omeprazole	20 mg	Take 1 capsule 30 minutes before breakfast once daily	PO	28 days / 28 capsules
Cetirizine	10 mg	Take 1 tablet once daily as needed for allergy symptoms	PO	30 days / 30 tablets

Medication orders above are fictional and included only to create test data for an OCR/NER/ICD-10/summarization system.

FOLLOW-UP / SAFETY INFORMATION

Follow up in approximately 4 weeks for review of home blood pressure readings and medication tolerance. This synthetic document does not provide individualized medical advice. Any real patient with concerning or worsening symptoms should seek professional medical evaluation.

CLINICAL NARRATIVE - ADDITIONAL DOCUMENTATION

This page intentionally contains context that should not all be treated as active diagnoses.

Review of Systems

System	Reported findings
Constitutional	No fever, chills, or unintentional weight loss.
Cardiovascular	No chest pain, orthopnea, paroxysmal nocturnal dyspnea, or syncope.
Respiratory	No shortness of breath or hemoptysis. No current wheezing.
Gastrointestinal	Intermittent reflux symptoms as described above. No hematemesis, melena, or persistent vomiting.
Genitourinary	No dysuria or hematuria. Reports occasional increased thirst.
Neurologic	No focal weakness, seizure, or loss of consciousness.
ENT	Intermittent seasonal sneezing and nasal congestion.

SOURCE / CODING NOTES FOR TESTING

1. Patient identity, dates, medical history, medication orders, laboratory values, and diagnosis statements in this file are entirely fictional.
2. The ICD-10-CM codes intentionally placed in the assessment section are: I10, E11.9, E78.2, K21.9, and J30.9.
3. The document contains both positive and negative clinical statements so that NER results can be evaluated separately from context/negation handling.
4. The document contains repeated medical terminology in sections such as family history and review of systems. A robust system should avoid turning every mention into an active patient diagnosis.
5. The main diagnosis/code table is deliberate ground truth for pipeline testing; the model should preserve the distinction between a stated diagnosis and a merely mentioned condition.

EXPECTED HIGH-LEVEL PIPELINE TARGET

Input	Fictional outpatient report PDF
Text extraction	All 4 pages + page boundaries
NER	Diseases, symptoms, medications, tests, findings and other biomedical entities
Context	Positive/negative/history/family-history/plan distinctions
ICD-10-CM	Map active documented diagnoses to the codes stated in the assessment
Summary	Human-readable explanation without introducing unsupported facts
JSON	Patient metadata + findings + entities + diagnoses + ICD codes + medication orders + provenance